

Please inform the patient if they are **ABCD 4 and above** that they will be contacted within 24 hours (weekdays) with an appointment in the rapid access clinic on ward 8

Patient Details:

Hospital Number:
Name: Mr Mouse Mickey
Address: St Chads, Cannock, WS7 0EW
Date of Birth: 10-Feb-2000
Telephone No:
Mobile Number:

GP Details:

Name: n
Surgery address:
Darwin Medical Practice
Greenwood Health Centre
Lichfield Road
Burntwood
Staffordshire WS7 0AQ
Tel: 01543 682654
Email: darwinmedical.practice@nhs.net

Risk Factors:

	Yes	No
Previous Stroke/Tia	☐	☐
Diabetes mellitus	☐	☐
Atrial fibrillation	☐	☐
Ischaemic heart disease	☐	☐
Smoker	☐	☐
Hypertension	☐	☐
Hyperlipidaemia	☐	☐
Alcohol intake	☐	☐

Current Medication:

Please see the attached list.

TIA ABCD Score

ABCD	RISK FACTOR	SCORE
AGE	Age below 60	0 ☐
	Age 60 or above	1 ☐
BLOOD PRESSURE	Systolic BP above 140 mmHg, and/or diastolic BP 90 mmHg or higher	1 ☐
CLINICAL	One-sided weakness of face, arm, hand or leg	2 ☐
	Speech disturbance without weakness	1 ☐
DURATION	Symptoms lasted more than 60 minutes	2 ☐
	Symptoms lasted 10 to 60 minutes	1 ☐
DIABETES	Patient is diabetic	1 ☐

Total ABCD score:

If score is 4 or above please treat patient as a **High**

Risk (See page 2)

What to refer:

Unilateral motor/sensory deficit
Hemiplegia or hemisensory syndrome
Sudden event of Speech or language difficulties
Amaurosis fugax
Homonymous hemianopia

What not to refer:

Migraine
Syncope
'funny turns' faintness
Dizziness Epilepsy/seizure
Transient global amnesia
Isolated confusion
Generalised weakness
Isolated vertigo, ataxia, tinnitus

Please organise the following investigations at the time of referral:

ECG
FBC
U&E
INR
Lipid profile
Blood Glucose

Clinical Details:

Commence Patient on aspirin 300mg until seen in clinic. ☐

Dypiridamole MR is now to be started after two weeks ☐

Referrer GP ☒ **A&E** ☐ **Paramedic** ☐ **Other** ☐ **Please specify:**

High Risk TIA Patients only

Patients that present within a week of their event who score 4 and above on the ABCD score should be treated as high risk of stroke and should receive treatment and investigation within 24 hours. Patients who present more than a week after their event should be treated as low risk. The clock starts at the time of their first contact with a health professional (Paramedic, nurse or GP) therefore rapid referral is essential. Please ensure date & time of first contact & referral are completed on page 1

For Clinic Use

Date & Time Referral received:

Were the following investigations completed within 24 hours of the first contact

Investigation	Notes	Completed within 24 hours of first contact	
		Yes	No
Blood Tests	All Patients	☐	☐
ECG	All Patients	☐	☐
Brain Scan *	If vascular territory or pathology uncertain	☐	☐
Carotid Imaging	Where indicated	☐	☐

* diffusion weighted MRI is preferred except where contraindicated, When CT should be used

Were the following treatments commenced within 24 hours of the first contact

Treatment	Notes	Completed within 24 hours of first contact	
		Yes	No
Aspirin or alternative	Unless contraindicated	☐	☐
Statin	Unless contraindicated	☐	☐
Control of BP	Where needed	☐	☐
Referral for carotid intervention	Where needed	☐	☐

NHS No:

Name: Mr Mouse Mickey (dob 10-Feb-2000)

Medication

Acute

Drug	Dosage	Last Issued On
Midazolam 5mg/5ml solution for injection ampoules	2.5-5mg 4 hourly as needed	09-Apr-2026
Morphine sulfate 10mg/1ml solution for injection ampoules	2.5-5 mg every 3-4 hours prn as needed max 30mg per 24 hour	09-Apr-2026
Folic acid 5mg tablets	One To Be Taken Each Day	
Rifampicin 300mg capsules	One To Be Taken At Night	
Isoniazid 50mg tablets	One Or Two To Be Taken At Night	
Sofradex ear/eye drops (Neon Healthcare Ltd)	Two Or Three Drops Instilled Into The Affected Ear(s)	
Nitrofurantoin 100mg modified-release capsules	Three Or Four Times Daily One To Be Taken Twice A Day	
Haloperidol 5mg/1ml solution for injection ampoules	1.5-2.5mg every 4 hours as needed	

Doxycycline 100mg capsules	Two To Be Taken On The First Day Then One To Be Taken Each Day
Colecalciferol 20,000unit capsules	take one capsule twice a week for 7 weeks then use an over the counter supplement which can be bought from supermarkets or chemists fro October to March
Hydroxocobalamin 1mg/1ml solution for injection ampoules	1ML
Shingrix (Herpes Zoster) adjuvanted recombinant vaccine powder and suspension for suspension for injection 0.5ml vials (GlaxoSmithKline UK Ltd)	When Required
Diamorphine 10mg powder for solution for injection ampoules	Anticipatory care as per community nursing authorisation and administration record for palliative care for pain if required
Hydroxocobalamin 1mg/1ml solution for injection ampoules	Immediately
Fortisip Compact liquid (Flavour Not Specified)	125ml twice a day
Fortisip Compact liquid (Flavour Not Specified)	125ml twice a day
Ciprofloxacin 0.3% ear drops	as directed
Midazolam 5mg/5ml solution for injection ampoules	2.5-5mg 4 hourly as needed
Diamorphine 30mg powder for solution for injection ampoules	Anticipatory care as per community nursing authorisation and administration record for palliative care for pain if required
Hydroxocobalamin 1mg/1ml solution for injection ampoules	Immediately
Salamol 100micrograms/dose inhaler CFC free (Teva UK Ltd)	One Or Two Puffs To Be Inhaled Up To Four Times A Day
Paracetamol 500mg tablets	One To Be Taken Every 4-6 Hours Up To Four Times A Day
Clarithromycin 500mg tablets	One To Be Taken Twice A Day
Humulin I KwikPen 100units/ml suspension for injection 3ml pre-filled pens (Eli Lilly and Company Ltd)	For Subcutaneous Injection, According To Requirements as per directive
Water for injections 10ml ampoules	2.5mg PRN for agitation
Midazolam 10mg/2ml solution for injection ampoules	As per directive
Morphine sulfate 10mg/1ml solution for injection ampoules	One To Be Taken Every 4-6 Hours Up To Four Times A Day
Paracetamol 500mg tablets	One To Be Taken Each Day
Folic acid 5mg tablets	4 months and then stop
Nitrofurantoin 100mg modified-release capsules	One To Be Taken Twice A Day
Morphine sulfate 10mg/1ml solution for injection ampoules	2.5-5 mg every 3-4 hours prn as needed max 30mg per 24 hour
Hydroxocobalamin 1mg/1ml solution for injection ampoules	1ML
Dressit sterile dressing pack with medium/large gloves (Richardson Healthcare Ltd)	10
Dihydrocodeine 30mg tablets	One To Be Taken Four Times A Day When Required
Phenoxymethylpenicillin 250mg tablets	Two To Be Taken Four Times A Day
Paracetamol 500mg tablets	Two To Be Taken Every 4-6 Hours Up To Four Times A Day

Haloperidol 5mg/1ml solution for injection ampoules

Water for injections 10ml ampoules

Ibuprofen 400mg tablets

Enoxaparin sodium 60mg/0.6ml solution for injection pre-filled syringes

Ciprofloxacin 500mg tablets

Estradiol 10microgram pessaries

Zostavax vaccine powder and solvent for suspension for injection 0.65ml pre-filled syringes (Merck Sharp & Dohme (UK) Ltd)

Folic acid 5mg tablets

Colecalciferol 20,000unit capsules

Doxycycline 100mg capsules

Anticipatory care as per community nursing authorisation and administration record for palliative care

Anticipatory care as per community nursing authorisation and administration record for palliative care

One or two To Be Taken Three Times A Day After Food as needed
As directed

One To Be Taken Twice A Day
10?micrograms daily for 2 weeks, then reduced to 10?micrograms twice weekly
1

One To Be Taken Each Day
4 months and then stop
take one capsule twice a week for 7 weeks then use an over the counter supplement which can be bought from supermarkets or chemists fro October to March

Two To Be Taken On The First Day Then One To Be Taken Each Day

Repeat

Drug	Dosage	Last Issued On
Atorvastatin 40mg tablets	One To Be Taken Each Day	25-Feb-2026
Topiramate 50mg tablets	One To Be Taken Twice A Day	11-Jan-2024
Gliclazide 80mg tablets	Two To Be Taken Daily With Breakfast	
Bempedoic acid 180mg tablets	One To Be Taken Once Daily	

Repeat Dispensing

Drug	Dosage	Last Issued On
Ramipril 1.25mg capsules	take one daily to lower blood pressure and protect heart	25-Feb-2026